RIP MEDICAL DEBT

September 11, 2023

Via Electronic Submission Only: www.regulations.gov

Czarina Biton, Team Lead, Marketplace Eligibility and Enrollment Group, Centers for Medicare & Medicaid Services

Octavian Carare, Supervisory Economist
Consumer Financial Protection Bureau

Thomas West, Deputy Assistant Secretary
U.S. Department of the Treasury

Re: Request for Information Regarding Medical Payment Products, Docket No. CFPB-2023-0038; Docket No. CMS–2023–0106, Docket No. TREAS–DO–2023–0008

Dear Mr. Carare, Ms. Biton, and Mr. West:

On behalf of RIP Medical Debt, I am pleased to submit the following comments regarding the **Request for Information (RFI) Regarding Medical Payment Products, Docket No. CFPB-2023-0038; Docket No. CMS–2023–0106, Docket No. TREAS–DO–2023–0008**. RIP Medical Debt is a national nonprofit organization committed to lifting the burden of medical debt for millions of people nationwide. Our unique model combines the generosity of donors with debt industry expertise to abolish medical debt, mitigating significant financial and mental distress for millions of people. Through our work, we also seek to bring attention to the negative impacts caused by medical debt, distill its causes through applied research, and ultimately use these findings to change public policies and drive practice improvements.

We appreciate your collective approach to the issues presented by medical payment products and welcome the opportunity to offer our insights and recommendations. We believe the growth of these products is one symptom of a broken health care system that leaves far too many people exposed to medical bills they cannot afford to pay. **In our current flawed health care marketplace, medical payment products may seem like the only recourse for people who are desperate to receive care. Yet the guardrails currently in place within the financial services and health care sectors are insufficient to protect patients from being directed towards high-interest credit cards and loans when more affordable options might exist.** Our comments aim to strike the appropriate balance between preserving health care access for patients trapped in an imperfect market and curbing the types of questionable and potentially usurious financial practices that have sparked this RFI. We urge the Department of the Treasury, the Department of Health and Human Services (HHS), and the Consumer Financial Protection Bureau (CFPB) to:

1. Strengthen existing health care safety net and financial protections for low- and moderate-income people;

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

2. Increase monitoring and public reporting about trends in medical debt and health insurance denials;
3. Work with members of Congress to expand affordable, comprehensive coverage and other consumer protections, such as financial assistance and fair billing practices; and
4. Invest in programs and technologies that help health care providers, community organizations, and financial services institutions inform and screen patients for financial assistance and coverage programs that will help them avoid debt.

.

I. Selected General Questions

A. Market-level inquiries

What are the benefits, costs, and risks of medical payment products for consumers, health care providers, and companies offering these products?

Our sense is that patients turn to riskier payment options such as credit cards when they perceive or believe that this is the best, or only, option they have for receiving health care services they need. **First, patients may feel compelled to pay with credit to avoid being refused care for unpaid bills.** Recent reporting of hospitals denying patients care when they have unpaid medical bills highlights how patients are vulnerable to a range of financing options in order to maintain health care access.[1] While it is not yet clear how widespread this particular practice is, having medical debt is itself a common phenomenon. A shows that nearly a quarter of middle-class families (22.5%) have medical debt despite having insurance coverage, along with 12.3% of higher-income families and 20.8% of lower-income families.[2] People with medical debt who still require health care may feel pressured to use credit to maintain continuity of care.

Second, health insurance has increasingly shifted costs to patients through benefit and payment designs that either do not include certain essential health benefits or that use high deductibles, coinsurance rates, and copays. According to research from Crowe Revenue Cycle Analytics, "the deductible amount and/or the residual amount due from the patient after insurance payment...represented nearly 60% of patient bad debt in 2021, a five-fold increase in just three years." Deductibles for people with employer-sponsored health coverage have skyrocketed, more than doubling since 2010. As the number of insurance products structured to include large out-of-pocket costs grows, it is more common for patients to be driven to strategies that rely on credit to cover their portion of the bill.

---

[1] See, e.g., Kliff, S. and Silver-Greenberg, J. "Nonprofit Health System Pauses Policy of Cutting Off Care for Patients in Debt," *New York Times* (June 9, 2023). Accessed September 11, 2023 at https://www.nytimes.com/2023/06/09/health/allina-health-nonprofit-medical-debt.html.

[2] See Murdock, K.; Kendall, J.; and Kendall, D. "Medical Debt Hits the Heart of the Middle Class." Third Way (August 21, 2023). Accessed September 11, 2023 at https://www.thirdway.org/report/medical-debt-hits-the-heart-of-the-middle-class.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

Similarly, patients may be susceptible to using credit or other risky payment products when their insurance plans carve out essential health services. This is particularly clear when looking at dental and audiology debt. These services are over-represented in the medical credit card market, likely due to the poor coverage offered by insurers in both public and employer-sponsored plans. For example, a recent survey by Kaiser Family Foundation found that about 7 in 10 Medicare beneficiaries report cost being a barrier to accessing hearing and dental services, often because the scope of their insurance benefit is so limited.[3] At the same time, KFF reports close to half of Medicare beneficiaries have hearing difficulties and nearly one-third have a visual impairment, meaning there is a high need for these services with little insurance coverage—making it a prime target for credit card financing.

How do patients faced with these scenarios react? It is true that some providers offer financial assistance, and that others may offer interest-free or low-interest payment plans to certain patients. **But if patients are not aware of or able to access these less risky alternatives, credit may seem to be the only feasible option other than forgoing or delaying care.** Patients who have access to credit may use their regular credit card to pay their out-of-pocket costs. If they can afford to pay down their credit card monthly, using credit to pay a copay or deductible is simply a matter of convenience. By contrast, patients without credit or with lower credit scores may be more likely to use a specialty medical credit card, installment loan, or crowdsourcing platform to finance their care.

Relying on credit to pay for health care exposes patients to certain risks. First, depending on the type of product offered, specialty medical payment products may make credit more accessible to people who otherwise would not qualify for a traditional card. But as CFPB has noted, these products may also place them at greater risk of default or expose them to higher interest rates and fees than traditional lines of credit. Second, patients who use credit to pay for medical bills have effectively "converted" their medical debt to credit card debt. What was once a zero-interest balance due to their medical provider may now be subject to high interest rates from their credit card company and subject to reporting to credit reporting bureaus. While the national credit reporting agencies have made changes to how medical debt is reported, patients who have used credit cards to pay their medical bills do not benefit from this protection. Third, patients who rely on credit cards to pay for health care services may be less likely to resolve inaccuracies in medical bills. Medical billing is highly complex and susceptible to error. Providers are working to resolve high rates of insurer denials and address coding errors that are inherent to the billing system all within a contracted time frame. Once a bill is moved onto a credit card and the provider is paid, it is more challenging for a patient to contest or request a correction.

[3] Freed, M., et al. "Dental, Hearing, and Vision Costs and Coverage among Medicare Beneficiaries in Traditional Medicare and Medicare Advantage." Kaiser Family Foundation (September 21, 2021). Accessed September 11, 2023 at https://www.kff.org/health-costs/issue-brief/dental-hearing-and-vision-costs-and-coverage-among-medicare-beneficiaries-in-traditional-medicare-and-medicare-advantage/.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

Patients can pay for care in many different ways, such as by medical credit card or loan, general purpose credit card, insurance, or through a zero-interest payment plan. What are the costs and benefits for health care providers of offering each of these methods? Are there situations where one method of payment is more advantageous than another?

Zero-interest payment plans can be a practical option for patients who might otherwise not be able to afford to pay their portion at the time of service. Having sliding scale, interest-free or low-interest payment plans available could provide an important lifeline to tide patients over during a health crisis. Some providers do offer zero- or low-interest payment plans (see one example here). However, interest-free options for providers often max out within six months from the time that the initial statement is sent. If the patient cannot afford the payment plan within this timeframe, providers may refer them instead to a bank loan offering or credit.

Specialty medical credit cards and installment loans offer providers the benefit of a quick payment in a system where an interest-free payment plan from a self-pay patient may take weeks, months, or longer to collect. Leveraging credit card options to maximize patient payment is one strategy providers can deploy to speed up receipt of the patient's portion of the bill. Credit cards do not require providers to be in the business of managing payment plans and servicing or supervising loans. As providers spend more time and resources on collection efforts, they are vulnerable to pitches from credit card companies that make it easier to resolve unpaid bills.

How do medical payment products and health insurance coverage interact? Do group health plan or health insurance issuer practices contribute to uptake of medical payment products by patients and providers?

As we stated above, we believe there are issues with benefit and plan design that contribute to the proliferation of medical payment tools in the health care marketplace. The increasing trend of people enrolled in high-deductible health plans, reported high rates of insurance denials, and an uneven landscape for financial assistance have created a complex maze patients are forced to navigate. Recent data highlights how this collision translates into medical debt burden for the middle class and the uninsured. We must also draw attention to how high rates of insurance denials combined with low rates of patient appeals have left people exposed to debts they may not even be responsible for—debts that many people turn to financing options to cover to continue accessing care. Insurer practices may elongate the billing process and increase pressure on patients to fulfill their perceived financial responsibility for unpaid medical bills.

What are some best practices for health care providers who offer medical payment products in avoiding adverse financial and health impacts for patients?

There are a number of best practices that health care providers can use to protect their patients from harm. Some of these practices include:

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

- **Committing to never charging interest** or to use products that charge no interest for outstanding medical debt**.** When a provider commits to not charging interest, the need for deferred interest products is eliminated.
- **Ensuring that financial assistance and affordable payment plans are mentioned first and in every subsequent request for payment**. Providers should take every opportunity to remind patients of their options for assistance, verbally and in billing statements.
- **Using presumptive eligibility tools to screen patients for eligibility for financial assistance and low-risk payment plans based on the patient's *ability*—rather than *propensity*—to pay.** Most outside companies that offer financing to patients can and do determine a patient's income, and therefore could also identify eligibility for financial assistance and some means-tested programs. Providers should only work with outside vendors that integrate eligibility screens for financial assistance with any payment tool they are offering.
- **Offering regular in-service and onboarding training to admissions and revenue cycle staff on financial assistance policies and protocols**. Health care admissions and revenue cycle jobs are high-turnover, high-volume positions that place a lot of demand on the employee. Providers should make sure employees understand and are able to clearly explain the assistance available to patients, as well as the terms and conditions for any payment products offered.
- **Establish guidelines for how medical payment products are marketed to patients**. This could include offering clear definitions of unfamiliar financial terms (such as deferred interest), requiring a patient first be presented with an opportunity to apply for financial assistance, and creating rules around when such products can be offered (e.g., never presenting a patient with a medical credit card application during an appointment).

What data collection, data analysis, and research actions should agencies take?

Currently there is no public reporting of medical debt data.[4] It is only when bills in collections are furnished to private credit reporting agencies and reported by those agencies that we have a snapshot—albeit an incomplete one—of how medical debt is acquired, where, and by whom. There is a pressing need to find alternative ways to gather data about medical debt, even when it is not included on credit reports. In the private sector, researchers and several business entities attempt to capture the prevalence of medical debt through surveys. However, these vary widely in the types of questions asked; the race, ethnicity, language and disability (REL-D) data collected; and whether they reflect a point in time or are longitudinal. **We recommend that the tri-agencies:**

Investigate how all-payer claims databases (and a federal pathway) could monitor medical debt through billing and payment data. Not knowing the prevalence of outstanding bills is a blind spot in monitoring medical debt. A national system that monitors billing and payment would help advance the work to address the inequities of medical debt. This includes debt in collections that are reported to

[4] The Census Survey of Income and Program Participation (SIPP) asks a question about medical debt (re-added in 2018) that provides a snapshot of medical debt but is one of the few federal efforts to monitor medical debt trends.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

bureaus but also debt in collections that is not reported, and late bills owed to the provider. Providers regularly work with third-party debt collection agencies on contract, rolling out debt files to those entities to collect on their behalf. That data could be shared with a repository to track unpaid medical bills and could assist in monitoring trends in patient's out-of-pocket payments, as well as utilization of charity care. Measuring these elements would be hugely beneficial to the public, government stakeholders, and providers, creating a new level of transparency around medical billing and medical debt.

Launch an advisory committee to develop a medical debt module to be added to the National Health Interview Survey (NHIS) and/or the Agency for Healthcare Research and Quality's Medical Expenditure Panel Survey (MEPS). The module can build off existing questions and provide deeper understanding of the prevalence and disproportionate harm of medical debt. Such a module could assist in capturing the growth trends and effects of medical debt on people including health equity; additionally, the module could include a range of questions about how people finance their health care, including the use of financial products. Any report should be disaggregated by race, ethnicity, language, disability, chronic illness, sex, gender, age, household, income and identity.

Ensure there are annual questions across various surveys (Federal Reserve, Labor, CDC, CMS, etc.) that address medical debt, payment method, insurance type/status and the intersection of social determinants of health and REL-D data. Including questions in annual surveys and reporting to the public are important surveillance tools in assessing the prevalence of medical debt, including regional variations. Per the Biden-Harris administration's April 2022 announcement on actions to protect consumers and lessen the burden of medical debt on American families, HHS is tasked with evaluating how medical billing affects patient access to affordable health *and* the accrual of medical debt. In addition to adding medical debt questions to the MEPS and SIPP surveys, HHS could add a state module to the Behavioral Risk Factor Surveillance System (BRFSS). The BRFSS is a helpful way to further explore the cyclical effects of medical debt on mental health and well-being.

Where medical payment products are causing harm, what are some specific levers for regulatory oversight and enforcement by Federal agencies that regulate financial products or health care providers? What complementary legislative actions are worth exploring? Where may additional statutory authority be needed?

First, more needs to be done legislatively to address the high out-of-pocket costs that leave people unable to pay their medical bills. Congress must act to better align the affordability standard with livable wages. Potential legislative solutions include prohibiting insurance premiums from being more than a certain percentage of an individual or family's income.

Second, Congress should create more financial incentives for dental provider participation in Medicaid. Congress has explored making dental services a permanent benefit for both Medicaid and Medicare. These changes are long overdue given the mounting evidence that dental care access can reduce chronic illness, improve life expectancy, and increase employment opportunities.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

Third, Congress should expand consumer protections that promote fair, transparent billing and collection practices—including financial assistance. As the CFPB has noted, the primary users of deferred interest medical credit cards are currently located outside of hospital settings. Yet, the only federal requirements that exist around financial assistance and billing apply to tax-exempt hospitals under Section 501(r) of the Internal Revenue Code.[5] The continued proliferation of for-profit hospitals means large swaths of the country are served by hospitals that have no meaningful requirements around financial assistance.[6] Additionally, many hospital financial assistance policies do not apply to separately owned physician or tertiary services (e.g. diagnostics, anesthesia, orthotics/prosthetics) that patients expect to access at a hospital. Congress should explore pathways to expand financial assistance and fair billing requirements to for-profit hospital facilities and to provider groups that offer services in hospital or hospital-like settings, like ambulatory urgent care facilities. Congress should also explore the need for additional enforcement strategies for noncompliance of Section 501(r).

Fourth, Congress should increase investments that support technology improvements for presumptive eligibility for financial assistance systems and consider matching funds for states that invest in systems that increase presumptive eligibility pathways for charity care. While a person can easily apply for a medical credit card (often being approved within the same day), it is much, much harder for the average patient to apply for financial assistance or Medicaid. Federal funding to support appropriate use of presumptive eligibility screening tools could assist rural and smaller hospitals, in particular, to screen patients for financial help. These entities often do not have the funds to invest in available software and rely largely on paper applications—and patients taking the time to complete them.

We have listed recommendations for Treasury, HHS and the CFPB in their sections below.

B. Individual Inquiries

We applaud the tri-agencies for asking for comment directly from patients who have used medical payment products. In response to the RFI, RIP conducted a brief survey of previous beneficiaries to get a better understanding of how individuals currently interact with medical credit cards and other specialty payment products. The survey was sent to a list of 1,087 individuals who have had medical debt abolished by RIP, and we received 90 total responses. Thirty-one percent of respondents (n= 29) to our

---

[5] See 26 U.S.C. § 501(r) and 26 C.F.R. § 1.501(r).

[6] As of 2021, just under 24% of hospitals in the United States were for-profit. However, the saturation of for-profit hospital care varies greatly across the country. To get a sense of the contrast, New York and Vermont have no for-profit hospitals, while over 50% of hospitals in Nevada and Texas are for-profit. Seventeen states and the District of Columbia exceed the national average. Unless a state has extended requirements for financial assistance to for-profit hospitals, patients in these states are not guaranteed access to hospital financial assistance. See "Hospitals by Ownership Type (2021)." Kaiser Family Foundation. Accessed September 11, 2023 at https://www.kff.org/other/state-indicator/hospitals-by-ownership/?dataView=1&activeTab=map&currentTimeframe=0&selectedDistributions=for-profit&sortModel=%7B%22colId%22:%22Location%22,%22sort%22:%22asc%22%7D. See also Kona, M. and Raimugia, V. "State Protections Against Medical Debt: A Look Policies Across US." Commonwealth Fund (September 7, 2023). Accessed September 11, 2023 at https://www.commonwealthfund.org/publications/fund-reports/2023/sep/state-protections-medical-debt-policies-across-us.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

survey reported being familiar with specialty medical payment products, which is in line with our sense that these products are an emerging—rather than widespread—issue. Survey respondents heard about these products through the following categories:

- Just under 50% (n= 11) learned about medical credit cards through either the hospital billing department or from their provider/their staff directly;
- 40% (n= 9) were presented information on medical payment products through the dentist's office;
- 35% (n= 8) reported learning about these products from other sources, varying from friends, the internet, and advertisement via mail.

Of particular concern to us was the fact most respondents were *not* informed of other payment options (82%, n= 19) before being presented with medical credit cards. Indeed, **only three people** reported being told about other methods of payment. While the actual uptake of these products was low (only 7 respondents report having used a medical credit card), attitude towards specialty medical payment products was split. It should be noted the same percentage (35%) felt very strongly that they were pressured into using these cards without fully understanding the consequences and in lieu of using traditional routes (i.e., filing with insurance, applying for financial assistance), so it may be that people who have used the cards have a more negative attitude than those who have not.

Most respondents familiar with the products did not feel they were presented as a good alternative to health insurance (65%, n= 15). However, a substantial portion of respondents did feel these products were presented as a viable alternative *instead of* billing insurance. Additionally, most respondents felt these products were *not* clearly explained and had limited understanding of how they work, including the interest rate, the meaning of the term "deferred interest" (perhaps the largest area of concern), and the length of time they had to make payments. **We feel this is an important area for more exploration: are the people who do use these cards more aggressively targeted than others? Are these cards more likely to be used when the way they work is misrepresented? What type of patient education should be required before signing up for these products? There is a clear need for more guardrails, particularly in the form of patient financial education and health care literacy.**

RIP asked beneficiaries who have used specialty medical payment products to share more about their experience and how the products were explained to them. While our survey response was small, people expressed a mix of relief at feeling like they had another option to pay for needed care and **distress at the fact these cards are "the best of a lot of bad options."** To quote one beneficiary:

> *Information on ways to help me pay for additional or adjusted costs was not readily forthcoming. Once it was decided I didn't have the means to pay, CareCredit was suggested, or maybe some type of payment plan with additional immediate partial payment. I was unprepared for the extra, felt anxious, frustrated, angry, and fearful they would not proceed.*

Other respondents shared that they are now in debt to CareCredit and are unable to continue getting important follow-up care until that debt is cleared. Additionally, it appears most of the people who

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

used these products were also insured and simply unable to bridge the gap between what their insurance covered and their out-of-pocket costs. As one respondent put it, "insurance is definitely not enough." While the overall numbers were small, we were concerned to find that the majority of people who were presented and used these cards (5 out of 7) had overdue bills sent to collections, sometimes incredibly quickly. Multiple people reported having bills sent to collections "within weeks" of a missed payment, while others reported being sent to collections within 6 months. One respondent shared they have been unable to find a place to live because of the impact of going to collections on their credit score.

When asked if they felt like using these products made their situation better, worse, or the same responses were split. Many people said they felt like using a medical credit card made their situation better "for an immediate solution" (i.e., they felt better because they were able to access much needed care), but others shared they had mixed feelings. To quote one respondent, "I was desperate, so it seemed fine at the time, but I'm in the same situation I am now when I first got it, **so in a way it's worse because I now don't have that option**." It appears to us that specialty medical payment products currently provide a quick and much-needed route to care that has become inaccessible due to high costs, confusing insurance plan designs, and difficulties accessing financial assistance. As one survey respondent said, **"I have no other way to pay the outrageous prices, but I don't like the idea of needing a credit card just to get medical/dental treatment."**

Overall, people expressed relief at feeling they had another option to pay for care but felt concerned that specialty medical payment products were the most accessible of multiple difficult options. The responses shared by our beneficiaries paint a picture of a health care system that is dense and difficult to navigate, with very few guideposts along the way. One respondent succinctly summarized what it is like to navigate the health care space:

> *This experience is so challenging. What you don't know to ask, where to/how to receive assistance, to be dependent on other people on the inside (so to speak) to willingly, forthrightly share information/point you in the right direction is ultimately frightening.*

As it stands, specialty medical payment products have the potential to be a valuable tool in easing the burden of accessing health care; however, without necessary guardrails like clear patient education, banning deferred interest, and other suggestions laid out in this document, these products have a high likelihood of making an already complex and intricate system to navigate even more difficult.

II. CFPB-Specific Questions

What actions should the CFPB consider taking to address problematic practices related to medical credit cards or loans, including debt collection and credit reporting practices?

The CFPB should ban financial institutions and vendors from offering and promoting deferred interest credit cards or loans in health care settings and for health care services, including dental and vision services; and limit the amount of interest that can be charged on elective procedures. The CFPB has

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

authority to address problematic practices related to medical credit cards or loans.[7] As advocates have said in their past comments, "Section 102(a) of the [Credit Card, Accountability, Responsibility and Disclosures Act of 2009] prohibits double-cycle billing, which is inherent in deferred-interest credit cards when consumers are subjected to a finance charge based on balances from prior billing periods if they do not pay their balance in full during the promotional period. In addition, Section 101(b) of the CARD Act prohibits the retroactive application of interest rate increases, which is also an inherent feature of deferred-interest credit cards."

Even if the CFPB does not instate a total ban on these products, it should limit how and where medical credit cards and installment loans can be marketed to patients. We are particularly concerned about point-of-service marketing in health care offices. The prospect of providers offering risky financial products to patients while they are in the middle of a health crisis is concerning. A patient who feels they must choose between a potentially predatory high-interest medical credit card and not receiving chemotherapy, for example, is not being presented with a fair choice. We believe guardrails are necessary to protect patients in these challenging situations to ensure providers are first offering patients less risky methods to pay down debt such as financial assistance for low-income and medically indigent patients, or zero-interest payment plans. Limitations on credit card marketing exist in other contexts. For example, tabling by credit card companies on college campuses has been forbidden. As a model, the CFPB should look at California Senate Bill 639 of 2019, which prohibits providers from arranging for or establishing an open-end credit or loan with a deferred interest provision.

We also believe that CFPB should require financial institutions and vendors that offer credit or loan products in medical contexts to provide evidence of "heightened informed consent" so that patients know all the potential financial side effects that come from signing up for a medical credit card. In clinical contexts, heightened informed consent is used for medical procedures deemed especially injurious or sensitive, such as sterilization or HIV testing. Because of the financial risks patients undertake with deferred medical credit cards, in particular, any financial services vendor that is offering these products should be required to demonstrate that they have helped patients understand the risks and that patients have first exhausted other options, including screening for coverage or charity care. Companies that offer credit or loan products in a health care context or for health services should be required to also give patients information about other options for paying for care, including notice about eligibility and application processes for public coverage programs, premium assistance, or financial assistance. People should also be provided a "cooling-off period" where they can retract their consent for these products while maintaining their access to care.

[7] 15 U.S.C. § 1637(j) (prohibiting double-cycle billing); 15 U.S. Code § 1666i–1 (prohibiting retroactive rate increases). See generally Wu, C. "Deceptive Bargain: The Hidden Time Bomb of Deferred Interest Credit Cards" (December 2015). National Consumer Law Center. Accessed September 11, 2023 at https://www.nclc.org/wp-content/uploads/2022/09/report-deferred-interest.pdf.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

Finally, **CFPB should work with HHS and Treasury to develop and implement data collection strategies for medical debt**, including medical debt on typical credit cards, so that the public can see and understand trends in medical debt at the individual and population level.

How can the CFPB use its authorities to ensure people with medical bills in collections, including medical payment product debt, are screened for eligibility for financial assistance and other benefits?

CFPB's own analyses show that the majority of medical credit products are offered in dental, visual, audiology settings, or with specialty providers who are often not bound by state or federal financial assistance, billing or collection requirements. That said, there is growing and valid concern that hospitals are leveraging medical payment products to address the accelerating issue of unpaid medical bills, increase revenues, and ease the burden of administering interest-free payment plans. We urge CFPB and the Internal Revenue Service (IRS) to collaborate and issue joint guidance reiterating that patients should be screened prior to any credit card or high-interest installment loan offer**.** Credit card companies are running income verification and should easily be able to identify income eligibility for a hospital's financial assistance policy. In addition, financial companies should ensure that if a person is found eligible for hospital financial assistance or interest-free payment plans after signing up for a medical credit card, they are able to extract themselves from their contractual obligation.

How are health care providers and financial companies using credit or "propensity to pay" scores to determine patients' eligibility for financial assistance or medical payment products? What are the implications for compliance with the Fair Credit Reporting Act or other CFPB authorities?

A growing number of companies sell revenue cycle tools that larger health care systems use to manage billing and collections. These companies, which include Experian, RevCo, Waystar and FinThrive,[8] offer software solutions that can help providers triage and screen patients for financial assistance, Medicaid, and other means-tested programs. These software solutions can also be used to help health care providers make decisions about where to focus collections activities. While the specific data inputs used for these software programs are not typically public, using algorithms to determine a patient's "propensity to pay"—that is, whether a patient is *likely* to pay their bill and not necessarily whether they are *able* to afford their bill—is a well-known strategy.[9]

[8] RIP Medical Debt partners with FinThrive to abolish medical debt.

[9] See, e.g., "Increasing Collections By Acting on Predictions of Propensity to Pay." HealthCatalyst. Accessed September 11, 2023 at https://www.healthcatalyst.com/success_stories/predicting-propensity-to-pay-allina-health#PREDICTING-PROPENSITY-TO-PAY. For a contrasting industry approach that delineates between ability to pay and propensity to pay, see also Gupta, C. "Propensity to pay vs. ability to pay: what's the difference?" PayZen (March 15, 2023). Accessed September 11, 2023 at Propensity to pay vs. ability to pay: what's the difference? – PayZen.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

Presumptive eligibility software could be a powerful tool to direct eligible patients into financial assistance and Medicaid programs, while minimizing effort from providers.[10] However, we are concerned that some providers and financial companies may be using "propensity to pay" scores inappropriately. Approaches that use a "propensity to pay" score instead of or prior to screening patients for hospital financial assistance or Medicaid eligibility—both of which are typically based on income—can penalize low- and middle-income people who have a history of paying their bills on time. **We urge CFPB to monitor revenue cycle products and, in partnership with HHS, to develop guidelines for health care providers and financial companies that use data analytics to streamline eligibility determinations for means-tested programs and medical payment products.**

When hospitals write off a patient's debt as uncollectible or "bad debt" and cease attempts to collect, do they notify patients that collection attempts will cease? Would patients benefit from such notifications, and would such notifications reduce hospital revenue?

A "bad debt" classification typically occurs after providers have attempted to screen patients for eligibility for Medicaid or their own financial assistance policies. Most hospitals do not inform patients whose accounts are classified as "bad debt" that attempts to collect will cease. The reasons for lack of notification are multifaceted. Some providers or collectors may desire to leave the door open for patients with "bad debt" accounts to make a payment in the future, even if this is statistically unlikely to occur. **Of greater concern, however, is the lack of notification to patients who have been found eligible for financial assistance through presumptive eligibility screening tools.** These patients are rarely informed that they have qualified for financial help and can still receive bills, even though the account has been classified as charity care. We encourage hospitals to notify patients when charity care – *whether full or partial* – is awarded.

III. HHS-Specific Questions

What actions should HHS consider taking to address problematic practices related to medical credit cards or loans, particularly as they relate to patients eligible for or enrolled in Medicare, Medicaid, or the Children's Health Insurance Program, or patients enrolled in Affordable Care Act Marketplace plans?

While recent improvements to ACA subsidies have made health insurance more affordable, too many people continue to struggle with high out-of-pocket costs. The questions raised in this RFI demonstrate the high level of complexity and financial literacy required to navigate our health care system. HHS should continue to invest in Navigators, restart investment in Consumer Assistance Programs (CAPs) and identify other assistance pathways that can help people avoid medical debt. This should include the launch of new learning modules for Navigators and other enrollment assisters about financial assistance

[10] See, e.g., _____. "The Patient as the New Payer: Five Opportunities to Improve the Patient Financial Experience" (suggesting providers use propensity-to-pay tools to route patients to financial assistance). Healthcare Financial Management Association (February 10, 2023). Accessed September 11, 2023 at https://www.hfma.org/revenue-cycle/patients-as-payers-five-ways-to-improve-the-patient-experience/.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

programs, consumer protections available to prevent medical debt, and other financial alternatives that can support low- and middle-income people who need help managing their medical bills.

In addition to supporting patient navigation and education, HHS should:

- **Analyze and report on the insurance denial rates for ACA, private and public plans to better understand how patients are financially impacted**. The ACA requires reporting by all non-grandfathered employer-sponsored health insurance plans and by non-group plans sold on and off the ACA marketplaces. Patients rarely appeal insurance denials and many just absorb the costs of care without question.[11] Data show that Medicaid insurers are routinely denying claims as part of their business practices, even when said claims should be covered. However, to date, HHS has arguably underused this data to provide oversight of insurance plans or to create market analyses consumers can use to select their health care plans.
- **Closely monitor billing practices for Medicaid patients and ensure they are protected from unnecessary credit card financing options.** Medicaid is prohibited from balance billing for services that Medicaid covers, meaning the provider cannot charge patients more than what Medicaid paid unless there is a written agreement with the patient to pay more than the Medicaid contracted rate or for a noncovered Medicaid service. The exception is for applicable Medicaid co-payments. Patients should regularly be informed of this protection.
- **Encourage the use of technology (such as presumptive eligibility systems) to automatically qualify eligible patients for financial assistance, Medicaid/CHIP, or Marketplace coverage.** Presumptive eligibility for hospital financial assistance as well as benefits programs can help significantly mitigate the harm of medical debt by guiding people towards the care and support they need.
- **Issue guidance on Title VI of the Civil Rights Act, clarifying that translation and interpreter requirements apply to financial conversations as well as clinical conversations.** Anecdotally, we have received stories of medical credit cards targeting individuals with limited English proficiency who did not understand they were signing up for a credit card—indeed, some people report thinking they were signing up for health insurance. In our recent comments to HHS on the Notice of Proposed Rulemaking for Section 1557 of the ACA, we argue that medical billing is a health care activity that should be subject to the same non-discrimination requirements that apply to clinical care.

Are there particular health care provider types that are most associated with being offered or offering medical payment products, and are these providers receiving directed payments or other incentives through State Medicaid programs?

[11]CMS data indicates that 17% of in-network claims were denied for plans purchased through healthcare.gov. The analysis highlights patients' low rates of appealing these denials: healthcare.gov patients appealed less 1% of denied in-network claims. See Pollitz, K., et al. "Claims Denials and Appeals in ACA Marketplace Plans in 2021." Kaiser Family Foundation (February 9, 2023). Accessed September 11, 2023 at https://www.kff.org/private-insurance/issue-brief/claims-denials-and-appeals-in-aca-marketplace-plans/.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

Evidence suggests these products are being used predominantly in primary and dental care settings—places that often serve as access points to other types of specialty care. It seems excepted benefits under the ACA (including adult dental, vision, and long-term care) are also common venues for these products. Since coverage is limited, patients in need of these benefits often face challenges in finding ways to pay—dental benefits are ripe for credit card offerings, particularly when insurance fails to cover anything more complex than cleanings. The ACA authorizes HHS to "periodically review the essential health benefits ...and provide a report to Congress and the public." In a letter to HHS in July 2023, health advocates highlighted that HHS has yet to conduct such a review and report to Congress. Numerous studies show the importance of dental care and its relationship to overall health status, and dental care is cited as the most commonly deferred service. Indeed, according to Kaiser Family Foundation, **about 4 in 10 people have delayed seeking medical and dental care due to cost.** HHS should take steps to review existing benefits and revise the EHB to include dental services to ensure people can afford these important services without incurring major medical debt.

How might HHS improve patient understanding of options for covering the cost of medical treatments? At what points in the care process could patients be provided with information about their financial obligations and payment options?

HHS could include the issue of how to pay for medical treatment in currently funded initiatives supporting the community and public health workforce. These multi-year grants already support training of community health workers, navigators, peer counselors and other non-clinical patient support roles. Curricula could include how to support clients with medical debt; how to apply for financial assistance or negotiate favorable payment plans with providers; and the role of medical debt as a social determinant of health and known barrier to care.

IV. Treasury-specific questions

What policy actions should Treasury consider taking to address problematic practices related to medical credit cards or loans, including debt collection and credit reporting practices, to conform with the existing tax laws and regulations pertaining to tax-exempt hospitals?

First, **Treasury should amend the regulatory definition of what constitutes a "reasonable effort"** to require hospitals and third-party vendors to conduct a full screening and eligibility determination for financial assistance *prior to* offering or directing patients to a medical payment product, including low-interest loans.[12] Steps required could include conducting a presumptive eligibility screen for financial assistance; communicating the outcome of that screening to the patient; and providing the patient with an opportunity and staff support to submit a full application for financial help if a presumptive screen proves inconclusive or negative.

[12] See 26 U.S.C. § 501(r)(6) and 26 C.F.R. § 1.501(r)-6, Billing and Collections.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

Second, Treasury should require hospitals to notify patients who have been found eligible for full financial assistance using presumptive eligibility tools. Currently, the Code requires hospitals that use presumptive eligibility tools to notify patients only when a presumptive screen finds the patient is either ineligible for financial assistance, or just qualifies for partial help.[13] It is silent about notice to patients who are found eligible for full free care through a presumptive screen. In response, most hospitals do not notify patients that receive full financial assistance through a presumptive screen. This creates uncertainty for patients, who may believe they still owe money. Indeed, some patients who have received financial assistance according to the hospital's records have reported that they still receive bills.

Should a tax-exempt hospital's signing patients up for medical payment products be considered similar to a tax-exempt hospital's selling medical debt, such that the special rules that only exclude debt sales from being extraordinary collection actions if certain requirements are met would be applied to these payment products?

We find it difficult to see how this exception would be workable or sufficiently protective for a patient that has signed up for a deferred interest credit card. This is one reason why we believe deferred interest medical credit cards should be banned outright. However, a narrow exception could arguably be made for interest-free or low-interest payment plans that 1) take into account a patient's ability to pay; 2) require the hospital and third-party vendor to both demonstrate that they have proactively screened the patient for eligibility for financial assistance and any coverage programs; 3) provide the patient with a meaningful opportunity to apply for financial assistance and coverage programs; and 4) allow for cancellation and reimbursement to the patient if the patient is later found to qualify for financial assistance.

How do tax-exempt hospitals' promotion of medical payment products compare to their operationalization of the requirement that their financial assistance policies be widely publicized?
The current requirements outline four key strategies tax-exempt hospitals must use to widely publicize their financial assistance policies.[14] Broadly speaking, hospitals must make information about financial assistance available online through the hospital's website; through paper copies available at the facilities; through broad community outreach; and by providing a plain language summary of the financial assistance policy directly to patients. In our experience, most tax-exempt hospitals meet the requirements related to posting information about financial assistance on their websites. While we do see hospitals commingling financial assistance with information about prompt-pay discounts or interest-free payment plans, in our experience it is not yet common for a tax-exempt hospital to advertise a medical credit card or high-interest bank loan.

It is less clear how hospitals are complying with the last three elements. Understanding how effective hospitals are at reaching patients and community members directly requires community insight,

---

[13] Ibid.

[14] See 26 U.S.C. § 501(r)(4) and 26 C.F.R. § 1.501(r)-4, Financial assistance policy and emergency medical care policy.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

knowledge, and access to patient billing materials.[15] One thing that does seem clear: despite these notice requirements, many patients are still unaware that financial assistance exists. In a survey report we conducted with nonprofit financial services provider Neighborhood Trust of beneficiaries and low to moderate-income workers, financial assistance is poorly understood and largely underutilized. The beneficiaries we surveyed routinely noted how difficult it is to get accurate, easily digestible information on their billing options and rights. Patients find themselves in complex medical situations with complex bills and must rely on overburdened billing staff to explain their options. Of note, **57% of respondents were not aware of financial assistance programs, over 44% of respondents never tried to apply for financial assistance, and 12% could not figure out how to apply**. To date, we are unaware of any large-scale public studies that have looked at how tax-exempt hospitals are implementing requirements to notify patients about financial assistance in their facilities, through direct communication, and through community outreach. This may be an area where Treasury can conduct or direct further study.

Treasury can also look to states that have instituted higher standards for notifying and qualifying eligible patients about hospital financial assistance.[16] For example, the state of Illinois requires hospitals to accept enrollment in several public programs as "presumptive eligibility" for financial assistance, including Medicaid, SNAP, WIC, and enrollment or referral from federally qualified health centers and community access programs that use income as an eligibility criteria. Illinois' approach illustrates the creative ways states and communities can leverage other means-tested programs to quickly move people to financial assistance efficiently and compassionately.

What are best practices for hospitals publishing and making patients aware of financial assistance programs (beyond compliance with the widely publicized requirements found in the section 501(r) regulations)?

RIP Medical Debt works with many hospitals that actively seek to improve how their financial assistance policies are implemented and shared with local communities. We encourage providers to make it as easy for patients to find out about and apply for financial assistance as it is for them to pay their bill. Good practices include having information about financial assistance no more than "two clicks" into the website. We also recommend providers include information about financial assistance in all patient

[15] One potential area for monitoring or improvement would be translation. 26 C.F.R. § 1.501(r)-4(b)(5)(ii) requires hospitals that serve patient groups over a certain size (1,000 people or 5% of the hospital's community, whichever is less) with limited English proficiency groups by translating the financial assistance policy, plain language summary, and application form into languages other than English. While it is common to find the plain language summary of a financial assistance policy translated into languages other than English on a hospital's website, it is rarer to find translated versions of the application and full policy.

[16] The Commonwealth Fund recently issued a helpful new survey of laws in all 50 states related to medical debt that compares existing state law to the requirements found in Section 501(r). Kona, M. and Raimugia, V. "State Protections Against Medical Debt: A Look Policies Across US." Commonwealth Fund (September 7, 2023). Accessed September 11, 2023 at https://www.commonwealthfund.org/publications/fund-reports/2023/sep/state-protections-medical-debt-policies-across-us.

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

billing statements. Any pull-out statements that direct patients to call the billing department to pay a balance or set up a payment plan can and should inform patients that they may be eligible for help.

To expedite the process for patients and staff, hospitals should adopt and implement a "no wrong door" approach to financial assistance. This requires hospitals to routinely offer updates and trainings to new hires and existing staff to ensure that everyone who interacts with patients—from social workers to nurses and clinicians, to financial services and vendor staff—know financial assistance is available and where they can direct patients to the appropriate personnel. Revenue cycle staff and vendors should be trained to screen patients for coverage options and financial assistance first, before offering a payment plan or financing product so patients have timely information about their financial options. Finally, we encourage our hospital partners to talk with their community partners. Local community partners have important lived expertise that allows them to better understand how accessible hospital financial assistance policies and application materials truly are, where they could be strengthened, and the best strategies for communicating directly with patients and the broader community.

Patients are inevitably going to fall through the cracks, which is why we encourage providers to think through improvements they can make to their financial assistance process—including automating the application process, reducing staff workloads and increasing the number of patients who can apply and get assistance. While these recommendations technically apply to other sections of 501(r), we include them here because we believe they are an important backstop to helping patients access financial help. They include:

- **Increasing hospital financial assistance eligibility levels to meet community need and expanding policies to include patients who are underinsured.** According to the American Hospital Association, a typical hospital financial assistance policy offers full free care to patients earning at or below 200% of the Federal Poverty Level (FPL).[17] Yet most of the debt we abolish at RIP Medical Debt is for patients with incomes between 150-350% of the Federal Poverty Level—including patients with insurance—and just over a quarter of all debt we abolish stemming from people whose incomes are between 150-200% FPL. This is a sizable gap between current hospital policies and community needs, particularly for people whose moderate incomes make them ineligible for a local hospital's financial assistance policy but who have high health care costs or struggle with underinsurance.[18] Currently their only recourse may be a loan, which can drain savings, or a credit card arrangement. In addition to adopting some of

[17] American Hospital Association, "Fact Sheet: Medical Debt" (June 2023) Accessed September 11, 2023 at https://www.aha.org/system/files/media/file/2023/06/fact-sheet-medical-debt.pdf

[18] The Commonwealth Fund defines underinsurance as having out-of-pocket costs, excluding premiums, that exceed 10% of household income in the previous 12 months for people over 200% FPL, or that exceed 5% of household income for people under 200% FPL; or having a deductible that exceeds 5% of household income. By this measure, approximately 23% of all American adults under the retirement age of 65 were underinsured last year. See, e.g. Collins, S., et al." The State of U.S. Health Insurance in 2022: Findings from the Commonwealth Fund Biennial Health Insurance Survey." Commonwealth Fund (September 29, 2022). Accessed September 11, 2023 at https://www.commonwealthfund.org/publications/issue-briefs/2022/sep/state-us-health-insurance-2022-biennial-survey

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

the best practices for screening and notification above, hospitals should revisit their eligibility levels.

- **Allowing interest-free payment plans for patients or guarantors below 400% FPL or who have at least 5% income tied up in medical debt.** Higher-income people also struggle with bills, and an interest-free or low-interest payment plan from their provider can be a better way to structure payment than credit. To work for patients, these payment plans should also be structured in installments the patient or guarantor can afford to pay.
- **Investing in technology that streamlines the financial assistance application process for patients and staff.** This includes 1) expanding the use of presumptive eligibility tools that can speed up a determination of eligibility for financial assistance or public coverage programs and 2) creating a platform that allows patients to submit and track their financial assistance applications online. This could be as simple as linking the financial assistance application to the BillPay section of a patient's online medical record, such as MyChart. Ideally these programs are used for every patient, not just uninsured patients, who may have difficulty paying a bill.
- **Removing questions about patient assets from financial assistance applications.** Applications for hospital financial assistance commonly ask patients to list assets like primary residences, cars, and various bank accounts, even though many eligibility determinations are based simply on a patient's or guarantor's income. Community partners and our beneficiaries have repeatedly shared examples of how including questions about assets in an application can have a chilling effect for patients who should have qualified for financial assistance, but never finished an application out of fear a key asset—their primary residence or car, retirement or education fund—would be seized. A better approach is to limit the application to information about patient income and medical indigence or underinsurance.
- **Removing requests for personal identifying information, like Social Security numbers (SSNs), or making them optional.** Many hospital financial assistance applications require patients to provide SSNs to aid staff in identifying whether the patient is eligible for Medicaid or other programs. Community partners have shared how this seemingly reasonable request can chill applications from people who lack an SSN. We recommend providers omit the SSN or, if including it, clarify on the form how it will be used (e.g., to screen patients for Medicaid)--and make it optional on the application form.

Does the availability of medical payment products generally benefit the community or assist patients financially?

Although medical payment products may assist certain patients, we do not believe it is appropriate for these products to be defined as "community benefit," even when they are offered at zero- or low-interest. First, community benefit should be reserved for financial assistance policies that absolve the patient of outstanding debt if they are unable to pay. Medical payment products, with the exception of zero-interest payment plans, generate profit for the financial services provider that now owns the debt. While providers may not be profiting from the exchange, treating these products as community benefit could further create perverse disincentives to enroll people who should be eligible for financial

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG

assistance for payment plans to increase revenue. Second, many of these medical payment products offer better terms, including lower interest rates, for people with better credit. While that might make perfect sense in the financial services sector, it is antithetical to the original purpose and intent of community benefit.

V. Conclusion

We are grateful for the intentionality with which the Treasury, HHS, and Consumer Financial Protection Bureau are approaching the issue of medical payment products. On behalf of our beneficiaries, thank you for your work. We look forward to next steps. Please reach out to Eva Stahl, Vice President of Public Policy (eva.stahl@ripmedicaldebt.org) with any questions.

Regards,

Allison Sesso

President & CEO, RIP Medical Debt

28-07 JACKSON AVE, 5TH FL, LONG ISLAND CITY, NY 11101 844-637-3328 WWW.RIPMEDICALDEBT.ORG